ClinicOS

An AI Clinic Operating System for Clinics and Hospitals, in India and Beyond

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India's digital health mission has registered around three and a half lakh clinics and hospitals. Sounds like progress. But only about four in ten are actually using any digital software on a normal day, and that's just the ones that got counted. Go to a small clinic in a district town, one doctor, no reception, and it's basically all paper.

That's basically the whole problem. Doctors across India, whether they're running one room alone or working inside a big hospital, don't have a simple way to turn a normal conversation with a patient into a proper written record. Most software out there is either too basic for a hospital or too heavy for a solo doctor with no IT team. Nothing really covers both.

Talking to doctors, a few things kept coming up. Writing notes takes time away from the next patient waiting outside. Handwritten prescriptions are hard to read, even for the doctor, let alone the pharmacist. More than once, doctors told me they wouldn't be comfortable with a system that keeps a permanent recording of what a patient says.

It's not that doctors don't want technology. It's that what's out there only really works at one end of the scale. It works best in English, not the Bengali and Hindi that doctors actually speak with patients. And it needs time nobody has.

That's the gap ClinicOS fills, for a solo clinic and a hospital ward alike. The doctor just talks, in whatever language feels natural, and the system turns that into a proper note, ready to check and sign off. Once signed, it becomes a printable prescription and gets saved to that patient's record. Here's the important part: the moment the doctor signs off, the recording gets deleted. Only the written note stays. That's built in, nobody has to switch it on, and it's a level of trust no platform that keeps every recording forever can offer.

We're building this step by step, testing with real doctors first. The scribe and patient timeline already work as prototypes. Chronic monitoring and specialist referrals come next.

There are real limits too. Once the system starts flagging health risks on its own, it's probably stepping into medical device territory, so that needs care. And because we delete the raw audio, we can't train a speech model on old recordings later. That's a trade-off we're okay with: trust matters more than convenience.

If this works, a doctor gets back the time they lose to handwriting, and a patient keeps a record that doesn't disappear after one visit, whether that's at a one-room clinic or a large hospital. This isn't meant to stay a small-clinic or India-only product. The same gaps show up everywhere, at every scale: no IT support, a language barrier, doctors who don't trust where their recordings end up. Put the features together and it's a genuinely better fit than what's out there today. Prove it here first, nationally and then beyond, and there's no real reason to stop.

Opening statistic: computed from the Ministry of Health & Family Welfare's Ayushman Bharat Digital Mission update (PIB, Dec 2024) — 1,52,544 of 3,49,473 facilities registered in the Health Facility Registry were actively using ABDM-enabled software (43.6%).